

Derbyshire Shared Care Pathology Guidelines

Post In-patient AKI management in primary care

Recommended actions to assess a patient following an episode of AKI:
(Timeliness of post-discharge care for adults following AKI is given on page 2)

1. Assess degree of kidney recovery

- Use serum creatinine at discharge and consider whether a repeat measure of kidney function is required for those patients who have not returned to their normal baseline kidney function – see page 2 for advice
- If a patient has new onset CKD following an episode of AKI assess and follow up as per CKD guidelines, which includes an assessment of albuminuria and a repeat serum creatinine at three months
- If you are concerned about a significant reduction in kidney function following an episode of AKI, then contact nephrology for advice

2. Review medications

Restart appropriate medications that may have been stopped during an AKI episode:

- Blood pressure tablets are often stopped but need restarting when BP rises during recovery
- ACEi/ARB and SGLT2 inhibitors can be restarted (unless specific advice to the contrary) once the kidney function has stabilised – U&E should be checked one week after reintroduction
- Cardiovascular risk: If aspirin (75mg once daily) and statins were stopped, these should be restarted unless specific reason not to. Aspirin 75mg is not nephrotoxic.
- If a drug has been specifically implicated in causing AKI (e.g. PPI leading to interstitial nephritis or NSAIDs), practice records should be updated to prevent the patient receiving these in future

3. Reduce risk of further AKI episodes in the future

- AKI sick day rules (see appendix for a patient leaflet)
- Avoid prescription of long term NSAIDs where possible, particularly in high risk patients and those with CKD
- Avoid prescribing triple combination of spironolactone, NSAID and ACEi/ARB
- Monitor kidney function after introducing certain medications:
 - ACEi/ARB/spironolactone
 - Trimethoprim and loop diuretics in patients with CKD

4. Code the occurrence of an AKI episode

using the specific read codes that currently exist (AKI 1, AKI 2, AKI 3)

Guidance on the timeliness of post-discharge care for adults following acute kidney injury (adapted from [AKI Post-discharge Timeline FINAL.pdf \(rcgp.org.uk\)](https://rcgp.org.uk))

Clinical Context At Point of Hospital Discharge				
AKI 3	Heart Failure + Poor Kidney Recovery* Consider clinical review by 3 days	No other significant factors* (No Heart Failure) + Poor Kidney Recovery*	Significant risk factors* (No heart failure) + Moderate Kidney Recovery*	No significant risk factors* + Moderate Kidney Recovery* Consider clinical review by 1 month
AKI 2	Heart Failure + Moderate or Good Kidney Recovery*			
AKI 1	Consider clinical review by 1-2 weeks	Consider clinical review by 1-2 weeks	Consider clinical review by 1-2 weeks	Significant risk factor* + Good Kidney Recovery* Consider clinical review by 1 month
				No Significant risk factor* + Good Kidney Recovery* Consider clinical review by 3 months
Blood Test	Consider U&Es by 1-2 weeks		Consider U&Es by 1 month	Consider U&Es by 3 months
Urine ACR	Consider Urine ACR by 3 months			

*KIDNEY RECOVERY

Consider the most recent stable creatinine prior to AKI to determine the degree of kidney recovery

- Good recovery
Creatinine ≤ 25% above baseline
- Moderate recovery
Creatinine >25% and <50% above baseline
- Poor recovery
Creatinine ≥ 50% above baseline

♦ SIGNIFICANT RISK FACTORS (IN ADDITION TO HEART FAILURE) PROMPTING EARLIER REVIEW

- Chronic kidney disease (CKD)
- Other cardiovascular risk factors: (diabetes, hypertension and established cardiovascular disease)
- Markers of vulnerability: (recurrent AKI, cancer treatment, sepsis, critical care)
- Markers of frailty: (those defined within the NHS England toolkit for general practice in supporting older people living with frailty)

Appendix: AKI sick day rules leaflet

Please open the attachment on the shared care pathology page

Other Useful Resources

- UK Kidney Association
[UK Kidney Association | The leading professional body for the UK kidney community](#)
This link includes many resources for healthcare professionals as well as patients.
- AKI Toolkit for GP's
<https://www.rcgp.org.uk/aki>
- NICE AKI guidelines (NG148) last update Oct 2024
<https://www.nice.org.uk/guidance/ng148>

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